INDEPENDENT REGULATORY COMPLIANCE ASSESSMENT

HOSPITAL PRICE TRANSPARENCY AUDIT

Independent Diagnostic Gap Analysis & Regulatory Exposure Assessment (45 CFR Part 180)

DATE: October 24, 2026

STANDARD: Hospital Price Transparency (45 CFR 180)

AUDITED HEALTHCARE FACILITY

Howard County General Hospital

Audited Source: 522093120_HowardCountyGeneralHospital_standardcharges.csv [->]

NON-COMPLIANT: DEFECTS DETECTED

Mandatory Statutory Remediation Required (45 CFR Section 180)

SCANNED ROWS	FLAGGED VIOLATIONS	SAFETY SCORE
13,376	2,119	84.7% (Grade B)
Machine-Readable CDM Records	Non-Compliant Data Coordinates	Compliance Benchmark (>=90%)

EXECUTIVE DIAGNOSTIC SUMMARY : STATUTORY FINDINGS

An algorithmic diagnostic inspection of the public Machine-Readable File (MRF) published by Howard County General Hospital revealed critical structural, syntax, and pricing irregularities that deviate from federal price transparency standards (45 CFR Section 180). The audit identified 2,119 compliance defects across 13,376 scanned chargemaster records, yielding a calculated statutory safety score of 84.7% (Grade B).

Hospitals are mandated to maintain published standard charge data that is complete, accurate, and free of placeholder records. The systemic defect patterns observed across this file leave the facility vulnerable to automated CMS crawler audits, mandatory Corrective Action Plans (CAP), and compounding daily civil monetary enforcement liabilities.

CMS SCHEMA INTEGRITY	PAYER RATE COMPLETENESS	ENFORCEMENT LIABILITY
CRITICAL DEFECTS	UNMAPPED / $0.00 RATES	ACTIVE CMP EXPOSURE
45 CFR Section 180.50	Standard Charge Completeness	45 CFR Section 180.90

PUBLIC AUDIT NOTICE: This independent technical diagnostic report evaluates publicly available hospital standard charge datasets pursuant to federal Hospital Price Transparency mandates (45 CFR Part 180).

SECTION I : REGULATORY RISK ASSESSMENT

Statutory Validation Failure & Enforcement Risk

Our automated Price Transparency Compliance Engine executed an algorithmic inspection of your facility's published Machine-Readable File (MRF). The published dataset failed statutory HPT V3.0 schema and data integrity benchmarks mandated under federal hospital price transparency rules.

STATUTORY CIVIL MONETARY PENALTIES (45 CFR SECTION 180.90)

The Centers for Medicare & Medicaid Services (CMS) strictly enforces MRF compliance through automated crawlers and targeted regulatory audits. Hospitals that fail to publish complete standard charge data face daily administrative fines scaled by licensed facility bed count:

Base Fine (<= 30 beds): $300 / day ($109,500 / year)

Scaled Fine (> 30 beds): $300 to $5,500 / day ($10 / bed / day) | Max: $2,007,500 / year

SECTION II : COMPLIANCE DEFECT LEDGER

Categorical Defect Breakdown

A granular inspection of your published dataset identified multiple categories of structural and pricing non-conformity. Missing gross charges, zero-dollar ($0.00) placeholder values, unmapped payer contracts, and schema omissions represent immediate points of regulatory exposure under federal price transparency mandates.

TOTAL DETECTED DEFECTS

2,119

Rule Violations

DEFECT FREQUENCY RATIO

0.16

Defect Triggers Per Inspected Row

STATUTORY ENFORCEMENT & ADMINISTRATIVE ESCALATION TIMELINE (45 CFR SECTION 180.70 - 180.90)

CMS administrative enforcement protocols triggered upon non-compliance detection:

1. Written Warning Notice | 30-Day Statutory Opportunity to Cure

CMS issues formal written notice specifying non-compliant chargemaster records and missing schema attributes.

2. Mandatory Corrective Action Plan (CAP) | Formal Remediation Protocol

Hospital must submit a detailed CAP within 30 to 90 days outlining technical remediation steps, validation gates, and milestones.

3. Civil Monetary Penalty (CMP) & Registry Notice | Compounding Daily Fines

Accrual of daily fines scaled to facility capacity ($300 to $5,500/day, up to $2,007,500/year under 45 CFR Section 180.90), combined with permanent public listing on the official CMS Hospital Price Transparency Non-Compliance Registry.

SECTION III : SAMPLE VIOLATION AUDIT

Verified Sample Violation Coordinates

Below are 5 verified sample error coordinates extracted from your published Machine-Readable File (MRF). Each entry identifies the exact source row index in your raw data file, the clinical procedure code (CPT/HCPCS), the violation taxonomy, and the statutory justification under Section 180.50.

ROW #	CPT / CODE	VIOLATION TYPE	DESCRIPTION & STATUTORY JUSTIFICATION
Row 13,112 MRF Record	**N/A** Scope	**Compliance Failure**	**Bilateral Procedure** Invalid Price Value: Missing required dollar standard charge where dollar amount is required by CMS v3.0 methodology.
Row 2,682 MRF Record	**N800** CPT / HCPCS	**Invalid Code Format**	**Clinical Diagnostic Lab Services** Billing code 'N800' does not match standard format rules for type 'APC'.
File Level Dataset Scope	**FILE_LEVEL** Scope	**Stale Date**	**Preamble update date is outdated** Last updated date '2025-07-01' is 480 days old (>365 days limit).
File Level Pre-Ingestion	**N/A** Scope	**Compliance**	**Invalid CMS Naming Convention** Filename 'temp_mrf.csv' does not follow CMS mandatory naming syntax (<EIN>_<hospital-name>_standardcharges.[csv|json]) per 45 CFR Section 180.50(d)(5).
Row 13,109 MRF Record	**N/A** Scope	**Invalid Clinical Setting**	**Setting value is missing or unpopulated (blank)** Missing required clinical setting value. CMS requires exactly one of: 'inpatient', 'outpatient', 'both'. Ref: 45 CFR Section 180.50(c)(2) & CMS Data Dictionary v3.0 Table 2.

ADDITIONAL FLAGGED VIOLATIONS IN ERROR LEDGER

The 5 coordinates above represent a diagnostic cross-section of your file's defects. A total of 2,114 additional non-compliant records (out of 2,119 total detected violations) are cataloged in the complete Line-by-Line Remediation Error Ledger (Remediation_Error_Ledger.csv). Each offline ledger record includes: exact raw row index, billing code (CPT/HCPCS/DRG/NDC), clinical description, payer contract tag, published price, and regulatory justification under Section 180.50.

CATALOGED DEFECTS	DELIVERABLE FORMAT	DIAGNOSTIC ATTRIBUTES
2,114 Records	**Standard CSV (.csv)**	**6 Diagnostic Fields**
In Error Ledger CSV	RFC 4180 Delimited Format	Row, CPT, Payer, Price & Justification

Data Governance Notice: Complete row-level defect workpapers are provided directly to hospital compliance leadership and are not published in public web indices.

SECTION IV : REMEDIATION WORKPAPERS

Technical Remediation Workpapers & Diagnostic Data Access

To remediate the compliance defects identified in this diagnostic audit and restore full statutory alignment, Elite Data Solution maintains complete line-by-line violation workpapers available for facility revenue cycle leaders and technical teams.

DELIVERABLE: COMPLETE LINE-BY-LINE REMEDIATION ERROR LEDGER

Remediation_Error_Ledger.csv

- Exhaustive line-by-line audit documenting all 2,119 detected violations
- Exact row coordinates mapped directly to your facility's published MRF line items
- Full clinical billing taxonomy (CPT, HCPCS, MS-DRG, NDC) & standard description fields
- Payer-specific contract discrepancy tags, zero-dollar rate flags, and statutory compliance citations
- Statutory 45 CFR Part 180 legal justification citing specific CMS price transparency mandates
- Standard RFC 4180 CSV export for hospital billing, IT, and revenue cycle analysis

DELIVERABLE FORMAT	AUDIT COVERAGE	COMPLIANCE STANDARD
CSV (.csv) / UTF-8	**100% Scanned (13,376 Records)**	**45 CFR Part 180**

TECHNICAL WORKPAPER REQUISITION & AUDIT CUSTODY

Authorized facility compliance officers and revenue cycle leaders may request formal transmission of workpapers:

Direct Outreach: audit@elitedatasolution.net **Web Portal:** https://elitedatasolution.net

Reference ID: [MRF Remediation Workpapers] - Howard County General Hospital

- **Transmission:** Delivered via email directly to authorized facility compliance leadership.
- **Defensibility:** Engineering workpapers structured pursuant to 45 CFR Part 180 statutory standards.
- **Confidentiality:** Defect workpapers are kept strictly confidential; restricted to authorized facility leadership.